InsightGlobal

HEALTHCARE AI & AGENTIC SOLUTIONS

Ideation case studies across the healthcare value chain

IGH PRESIDENT BRIEFING · NOVEMBER 2026

Agenda

1. Who we are
2. Our approach
3. Care Engagement
4. Life Sciences
5. Pharmacy & Drug
6. Medical Devices & IoT
7. Manufacturing & Supply Chain
8. How we engage

A core team built for any healthcare requirement

From strategy to production across AI, data, Salesforce and cloud, with healthcare compliance built in.

Agentic AI & GenAI engineering

Multi-agent workflows, RAG / GraphRAG, evaluation and guardrails, model-agnostic.

Knowledge graph & data engineering

Healthcare ontologies (RxNorm, ICD-10, SNOMED, LOINC, NPI, UDI), ETL, vector search.

Salesforce for healthcare

Health Cloud, Life Sciences Cloud, Marketing Cloud, Data Cloud, Agentforce, MuleSoft.

Multi-cloud delivery

Google Cloud, AWS and Azure. The same design runs wherever the client already is.

Healthcare compliance & governance

HIPAA, GxP / Part 11, FDA device and safety rules, consent, masking and audit trails.

Product & delivery

Ideation workshops, architecture, proof of concept to production, change management.

PROOF POINT We built a governed agentic delivery platform for a large US health system (Salesforce Health and Marketing Cloud, masked data context, compliance guardian, human approval gates).

Our core team

[Name]
Engagement lead

[Name]
AI / agentic architect

[Name]
Knowledge graph & data lead

[Name]
Salesforce Health Cloud architect

[Name]
Marketing & Data Cloud lead

[Name]
Cloud & platform engineer

[Name]
Healthcare compliance lead

[Name]
Clinical / domain SME

[Name]
Delivery manager

One reusable pattern, applied to every domain

Every case study: problem → solution → architecture → value

1

Data sources

Public reference data + client systems (EHR, CRM, QMS, devices)

2

Ingest & govern

ETL, de-identification, masking, consent, data quality

3

Knowledge layer

Knowledge graph + vector search + masked analytics

4

AI agents

Specialist agents that plan, cite and hand off to people

5

Experience

Salesforce, portals, Tableau, existing workflows

Governance built in: guardrails in and out · minimum-data access · human approval gates · evaluation sets · full audit trail

Runs on any cloud

Google Cloud | AWS | Azure | Salesforce

Where we play across the healthcare value chain

ENGAGE & DELIVER CARE

01

Care Engagement

- **A** Care-gap closure & journeys
- **B** Access & financial navigation

Buyers: Health systems, provider groups, health plans

DEVELOP & COMMERCIALIZE

02

Life Sciences

- **A** HCP engagement & MLR pre-review
- **B** Trial feasibility & recruitment

Buyers: Pharma, biotech, CROs, academic medical centers

DISPENSE & MANAGE MEDICATIONS

03

Pharmacy & Drug

- **A** Med reconciliation & interaction safety
- **B** Specialty Rx hub & prior auth

Buyers: Health-system pharmacy, specialty pharmacy, PBMs, pharma patient-services hubs

MONITOR DEVICES & PATIENTS

04

Medical Devices & IoT

- **A** Device complaints & surveillance
- **B** Remote patient monitoring

Buyers: MedTech manufacturers, health systems, remote-monitoring providers

MAKE & SUPPLY

05

Manufacturing & Supply Chain

- **A** Deviation & CAPA copilot
- **B** Shortage early warning

Buyers: Pharma and biotech manufacturing, CDMOs, distributors, hospital pharmacy

01

CARE ENGAGEMENT

A · Intelligent care-gap closure & patient journeys

B · Patient access & financial navigation agent

Who buys: Health systems, provider groups, health plans

Intelligent care-gap closure & patient journeys

THE PROBLEM

Health systems run many outreach campaigns but can't see why a group of patients has a care gap, which intervention works for whom, or whether outreach is fair across communities.

Who feels it: Population health, care coordination, marketing ops, quality teams

- Care gaps and outreach live in separate systems
- Same message to everyone, so low conversion and opt-outs
- Privacy rules slow every new campaign

27%

better patient adherence reported by Salesforce Health Cloud customers

Source: Salesforce (vendor-reported)

OUR SOLUTION

Specialist AI agents, grounded in trusted data, with people in control

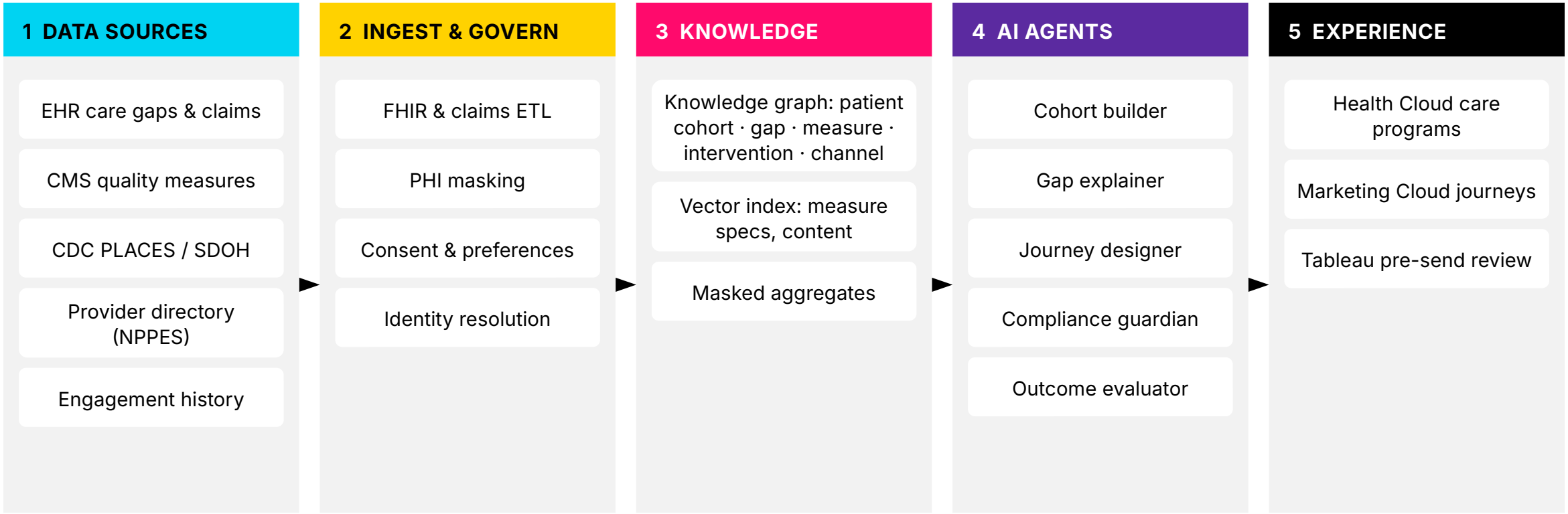
- Cohort agent builds patient groups from plain-English questions and returns masked counts, never raw patient data
- Gap explainer links quality measures, social drivers of health and access to explain each gap
- Journey designer drafts personalized Marketing Cloud journeys by channel, timing and content
- Compliance guardian checks consent, HIPAA and TCPA rules before anything is sent

BUILT ON

Health Cloud | Marketing Cloud | Data Cloud | Tableau

HIGH-LEVEL ARCHITECTURE

How it works: Care-gap closure & journeys



GOVERNANCE Guardrails in & out · PHI masking & minimum-data access · Human approval gates · Evaluation set · Audit trail

Runs on any cloud: Google Cloud · AWS · Azure · Salesforce

○ Yellow outline = human in the loop

VALUE & BENEFITS

Close more care gaps, with outreach that's personal, fair and compliant.

↑

Gap-closure rate per campaign

Pilot target

↓

Days to launch a compliant campaign

Pilot target

29%

faster case resolution reported with Health Cloud

Source: Salesforce (vendor-reported)

100%

of sends checked for consent and PHI before release

Design guarantee

Business

Quality-measure revenue, fewer no-shows, better retention

Clinical

Earlier screening and chronic-care follow-up

IT & data

One governed data layer reused across campaigns

Compliance

Minimum-data access, every send auditable

Patient access & financial navigation agent

THE PROBLEM

Patients drop out when they can't find the right provider, an open slot or a price they understand. Price-transparency files are public but nearly impossible for patients to use.

Who feels it: Patient access centers, revenue cycle, digital front door teams

- Long call-center queues for simple scheduling and price questions
- Estimates are slow, inconsistent or missing
- Patients book outside the network

Since 2021

US hospitals must publish machine-readable price files (CMS price transparency rule)
Source: CMS

OUR SOLUTION

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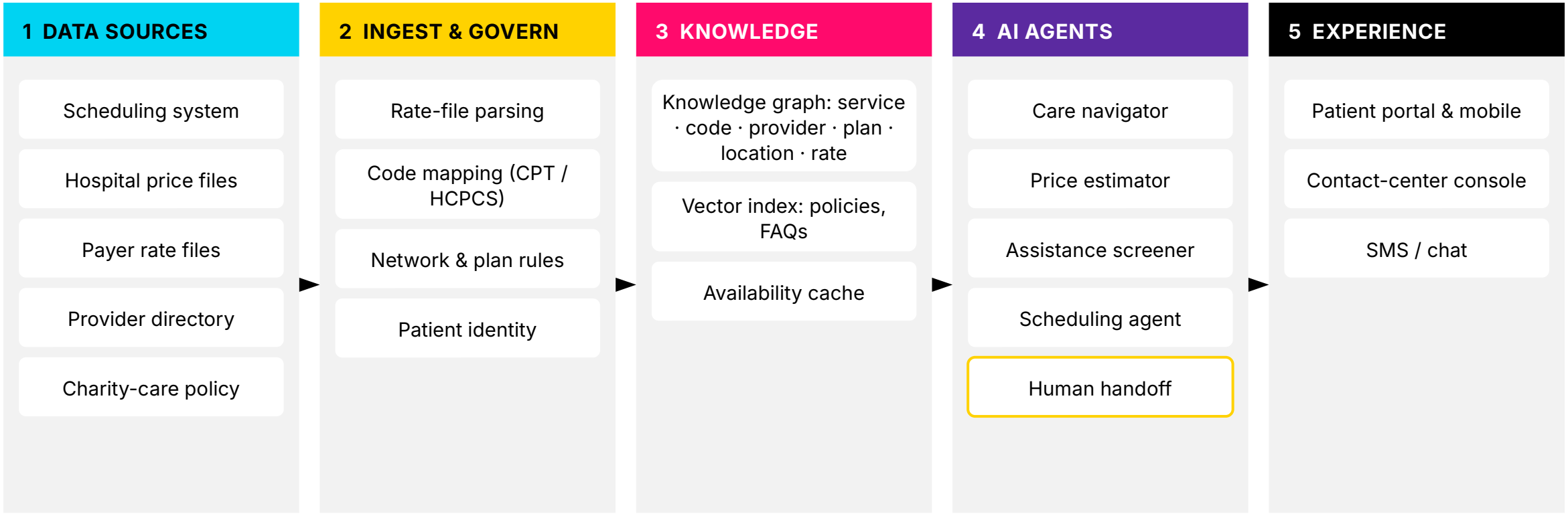
- Care navigator matches a symptom or service to the right in-network provider and open slot
- Price estimator combines price-transparency files and plan rates into a plain-language estimate
- Financial-assistance screener checks eligibility and starts the application
- Agent hands complex cases to a human with the full context

BUILT ON

Health Cloud | Experience Cloud | Agentforce | MuleSoft

HIGH-LEVEL ARCHITECTURE

How it works: Access & financial navigation



Runs on any cloud: Google Cloud · AWS · Azure · Salesforce

Yellow outline = human in the loop

VALUE & BENEFITS

Turn the digital front door into booked, in-network, financially clear visits.

↑

Online booking conversion

Pilot target

↓

Calls per booked appointment

Pilot target

↑

In-network referrals kept

Pilot target

24/7

self-service scheduling and estimates

Design capability

Business

More booked visits, less leakage, lower call volume

Clinical

Patients reach the right care faster

IT & data

Reuses directory and scheduling data already in place

Compliance

Supports price-transparency and No Surprises Act goals

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02
LIFE SCIENCES

A · Compliant HCP engagement & MLR pre-review

B · Clinical trial feasibility & patient recruitment

Who buys: Pharma, biotech, CROs, academic medical centers

Compliant HCP engagement & MLR pre-review

THE PROBLEM

Brand teams want a 360° view of healthcare professionals (HCPs) and next-best-action guidance, but every message must stay on-label and pass Medical-Legal-Regulatory (MLR) review, which takes weeks.

Who feels it: Brand and omnichannel teams, field reps, Medical Affairs, MLR reviewers

- HCP data is spread across CRM, claims and public sources
- Generic content gets low engagement
- MLR review is a bottleneck for every asset

$14.67B

in industry payments to HCPs across 17M records published by CMS Open Payments (Program Year 2025)

Source: CMS via IntuitionLabs

OUR SOLUTION

Specialist AI agents, grounded in trusted data, with people in control

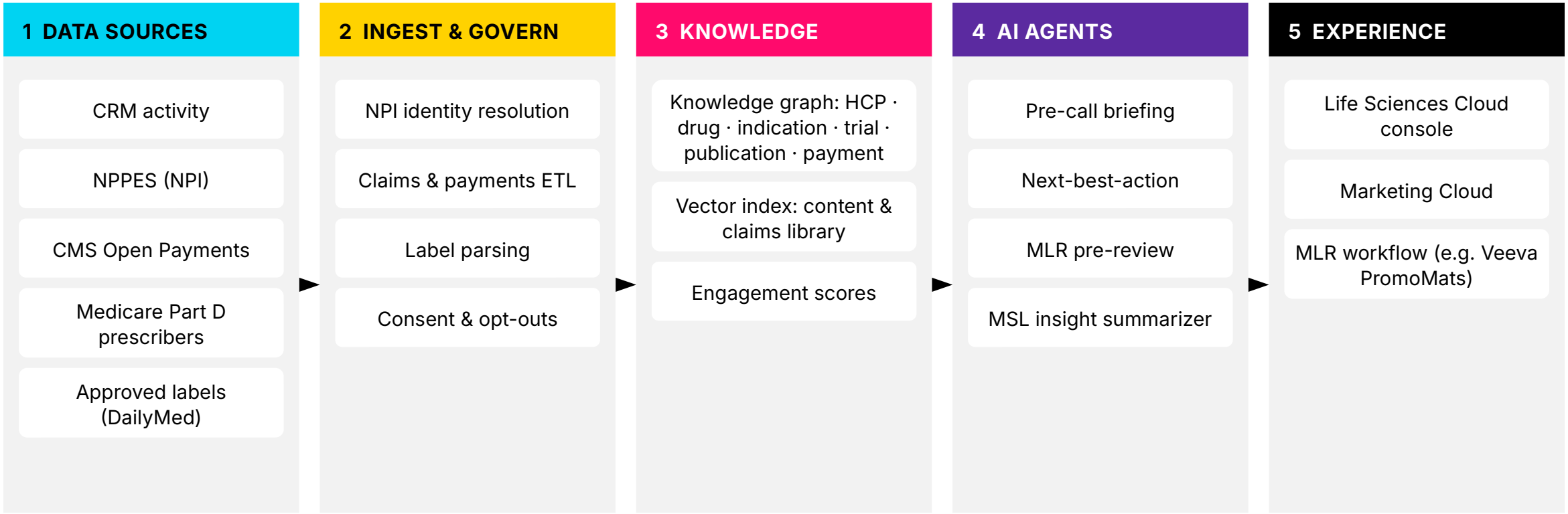
- HCP 360 links prescribing, payments, trials and publications by NPI
- Pre-call briefing agent gives reps a one-page, compliant summary
- Next-best-action agent suggests channel, content and timing
- MLR pre-review agent flags off-label claims against the approved label before human review

BUILT ON

Life Sciences Cloud | Marketing Cloud | Data Cloud | Agentforce

HIGH-LEVEL ARCHITECTURE

How it works: HCP engagement & MLR pre-review



GOVERNANCE Guardrails in & out · PHI masking & minimum-data access · Human approval gates · Evaluation set · Audit trail

Runs on any cloud: Google Cloud · AWS · Azure · Salesforce

○ Yellow outline = human in the loop

VALUE & BENEFITS

More relevant HCP engagement, and faster, safer content approval.

↓

MLR cycle time per asset

Pilot target

↓

Content rejected for off-label claims

Pilot target

↑

HCP engagement rate

Pilot target

Oct 2025

Life Sciences Cloud for Customer Engagement went GA

Source: Salesforce

Business

Faster launches, better rep productivity

Clinical

Accurate, on-label information reaches HCPs

IT & data

One HCP graph across brands

Compliance

Sunshine Act and PhRMA Code guardrails built in

Clinical trial feasibility & patient recruitment

THE PROBLEM

Most trials miss enrollment timelines. Eligibility criteria are free text, sites are chosen on relationships, and coordinators screen patients by hand.

Who feels it: Clinical operations, feasibility teams, site networks, CRO partners

- Manual chart screening against long criteria lists
- Site selection not grounded in patient data
- Under-representation of diverse patients

~40% less screening time with NIH's TrialGPT at matching accuracy close to experts (87.3%)
Source: NIH, Nature Communications 2024

OUR SOLUTION

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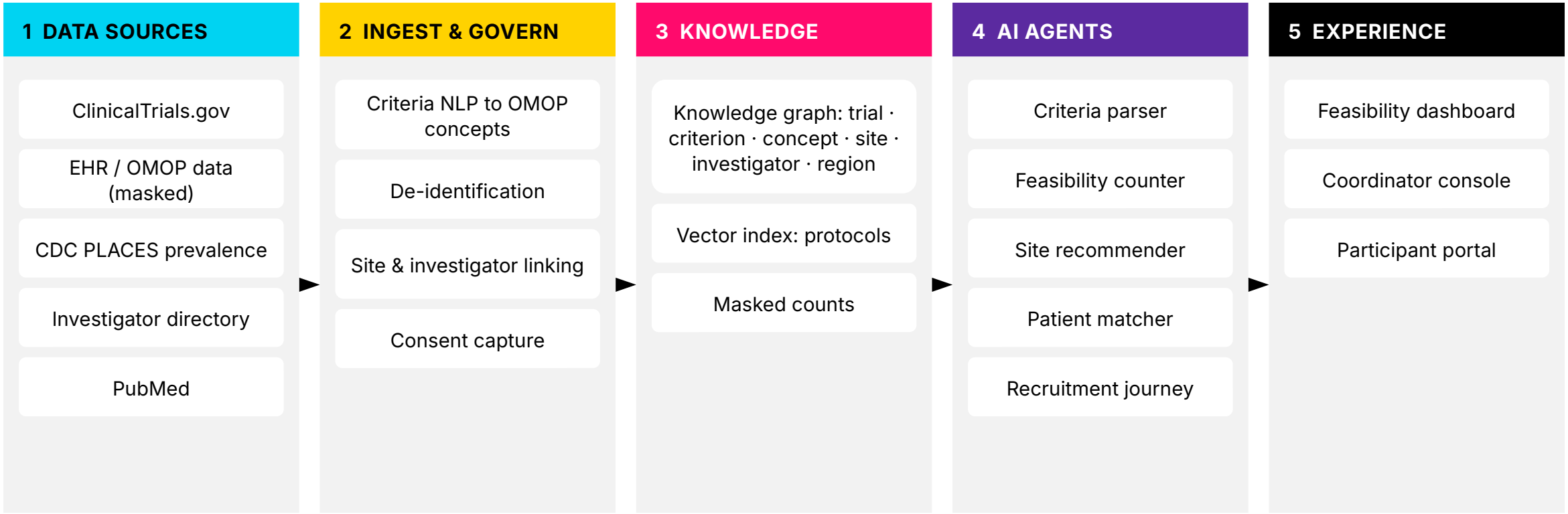
- Criteria parser turns free-text eligibility into coded concept sets
- Feasibility agent returns masked patient counts by site and region
- Site recommender ranks sites by patient pool, past performance and diversity
- Patient matcher explains each criterion; recruitment journeys run with consent

BUILT ON

Life Sciences Cloud | Marketing Cloud | Experience Cloud

HIGH-LEVEL ARCHITECTURE

How it works: Trial feasibility & recruitment



Runs on any cloud: Google Cloud · AWS · Azure · Salesforce

Yellow outline = human in the loop

VALUE & BENEFITS

Find the right sites and patients faster, with every match explained.

~40%

less clinician screening time (TrialGPT)

Source: NIH 2024

87.3%

patient-to-trial matching accuracy (TrialGPT)

Source: NIH 2024

↓

Time to first patient in

Pilot target

↑

Enrollment diversity

Pilot target

Business

Shorter trials, earlier revenue, fewer rescue sites

Clinical

More patients offered relevant trials

IT & data

Reusable criteria and site graph across studies

Compliance

Masked counts only; IRB-approved outreach

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03
PHARMACY & DRUG

A · Medication reconciliation & interaction safety

B · Specialty pharmacy hub: benefits, prior auth & adherence

Who buys: Health-system pharmacy, specialty pharmacy, PBMs, pharma patient-services hubs

Medication reconciliation & interaction safety

THE PROBLEM

Medication lists are messy: free text, brand versus generic, duplicates. Pharmacists reconcile them by hand, and general-purpose AI models miss dangerous interactions.

Who feels it: Hospital pharmacists, transitions-of-care nurses, ambulatory clinics

- Time-consuming reconciliation at admission and discharge
- Interaction alerts are noisy, so real risks get missed
- Adverse drug events drive readmissions

4 of 4

AI models tested missed the metoprolol–verapamil interaction in a 2024 study, which is why answers must be grounded

Source: ERCSP 2024

OUR SOLUTION

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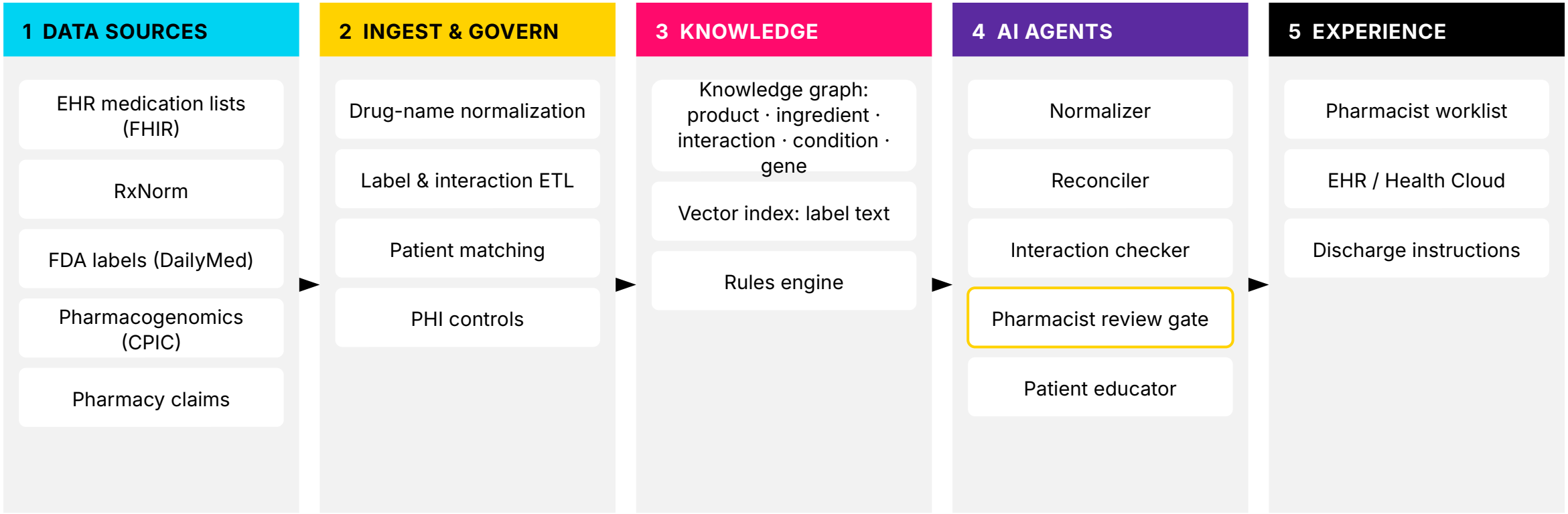
- Normalizer maps any free-text drug name to a standard drug code (RxNorm)
- Reconciler compares home, admission and discharge lists and flags discrepancies
- Interaction checker uses a curated drug knowledge graph first; AI only explains
- Pharmacist approves; patient-friendly instructions generated at the right reading level

BUILT ON

Health Cloud | EHR integration (FHIR) | Agentforce

HIGH-LEVEL ARCHITECTURE

How it works: Med reconciliation & interaction safety



Runs on any cloud: Google Cloud · AWS · Azure · Salesforce

Yellow outline = human in the loop

VALUE & BENEFITS

Safer medication lists in minutes, with every alert traceable to its source.

↓

Pharmacist minutes per reconciliation

Pilot target

↑

Discrepancies caught before discharge

Pilot target

↓

Readmissions linked to adverse drug events

Pilot target

100%

of interaction answers cited to a source

Design guarantee

Business

Fewer readmissions and penalties, pharmacist capacity freed

Clinical

Fewer adverse drug events

IT & data

Standard drug codes reused across apps

Compliance

Human verification on every change

Specialty pharmacy hub: benefits, prior auth & adherence

THE PROBLEM

Specialty drugs need benefits checks, prior authorization, copay help and adherence support. Each step is a manual handoff, so patients wait weeks or abandon therapy.

Who feels it: Pharma patient-services hubs, specialty pharmacies, provider prior-auth teams

- Prior-auth packets assembled by hand from charts
- Denials and appeals drafted from scratch
- Adverse events in support calls can be missed

Jan 2027

CMS-0057-F requires impacted payers to offer a FHIR prior-authorization API (most requirements from Jan 1, 2027)
Source: CMS Interoperability & Prior Authorization Final Rule

OUR SOLUTION

Specialist AI agents, grounded in trusted data, with people in control

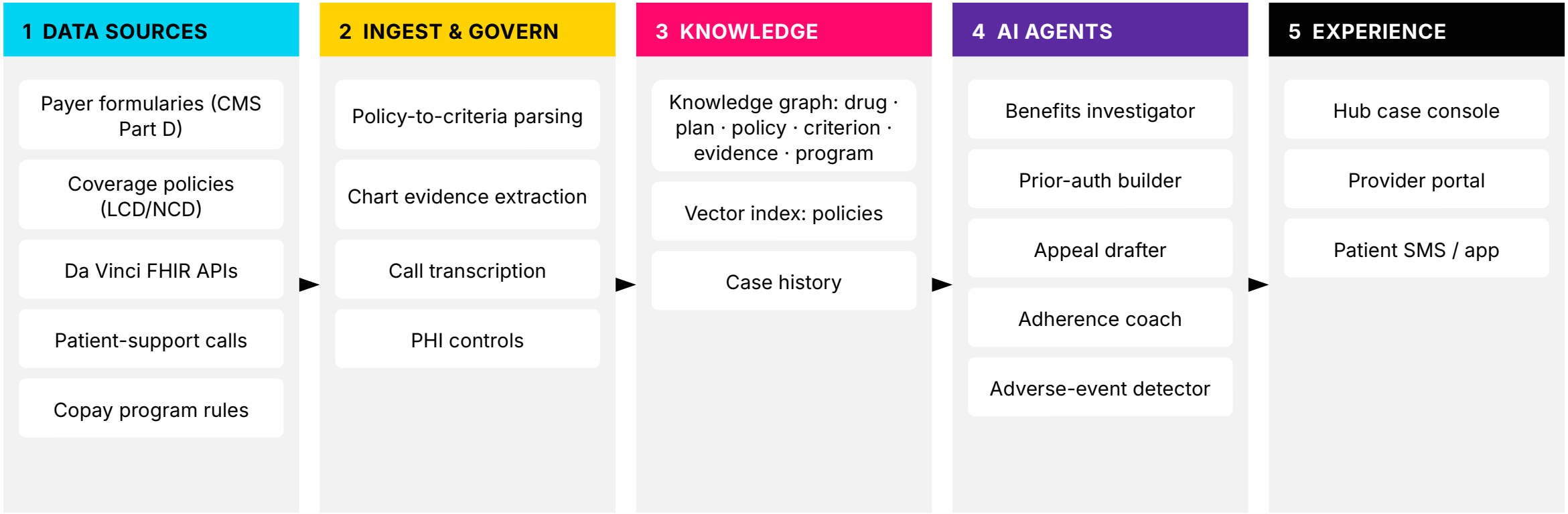
- Benefits investigator checks coverage, formulary tier and requirements
- Prior-auth builder maps chart evidence to each payer criterion, with citations
- Appeal drafter and copay-program matcher remove manual rework
- Adherence coach nudges refills; adverse-event detector flags safety reports in calls

BUILT ON

Life Sciences Cloud / Health Cloud | Service Cloud | Marketing Cloud | Agentforce

HIGH-LEVEL ARCHITECTURE

How it works: Specialty Rx hub & prior auth



GOVERNANCE Guardrails in & out · PHI masking & minimum-data access · Human approval gates · Evaluation set · Audit trail

Runs on any cloud: Google Cloud · AWS · Azure · Salesforce

○ Yellow outline = human in the loop

VALUE & BENEFITS

Get patients on therapy faster and keep them there.

↓

Days from prescription to first fill

Pilot target

↑

Prior-auth first-pass approval

Pilot target

↑

Proportion of days covered (adherence)

Pilot target

Jan 2027

CMS prior-authorization API deadline drives demand now

Source: CMS-0057-F

Business

Less abandonment, more patients on therapy

Clinical

Faster access to specialty treatment

IT & data

FHIR-ready for the CMS prior-auth APIs

Compliance

Adverse events captured and routed to drug safety

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04
MEDICAL DEVICES & IOT

A · Complaint handling & post-market surveillance

B · Remote patient monitoring & connected devices

Who buys: MedTech manufacturers, health systems, remote-monitoring providers

Complaint handling & post-market surveillance

THE PROBLEM

Device makers must triage every complaint, decide whether it's reportable to FDA, code it consistently and spot trends early. Today that's manual, slow and inconsistent.

Who feels it: Complaint handling, post-market surveillance, regulatory affairs, quality

- Complaint backlogs and late FDA reports
- Inconsistent adverse-event coding
- Trends spotted after a recall, not before

2M+

medical device reports received by FDA each year (MAUDE)

Source: FDA

OUR SOLUTION

Specialist AI agents, grounded in trusted data, with people in control

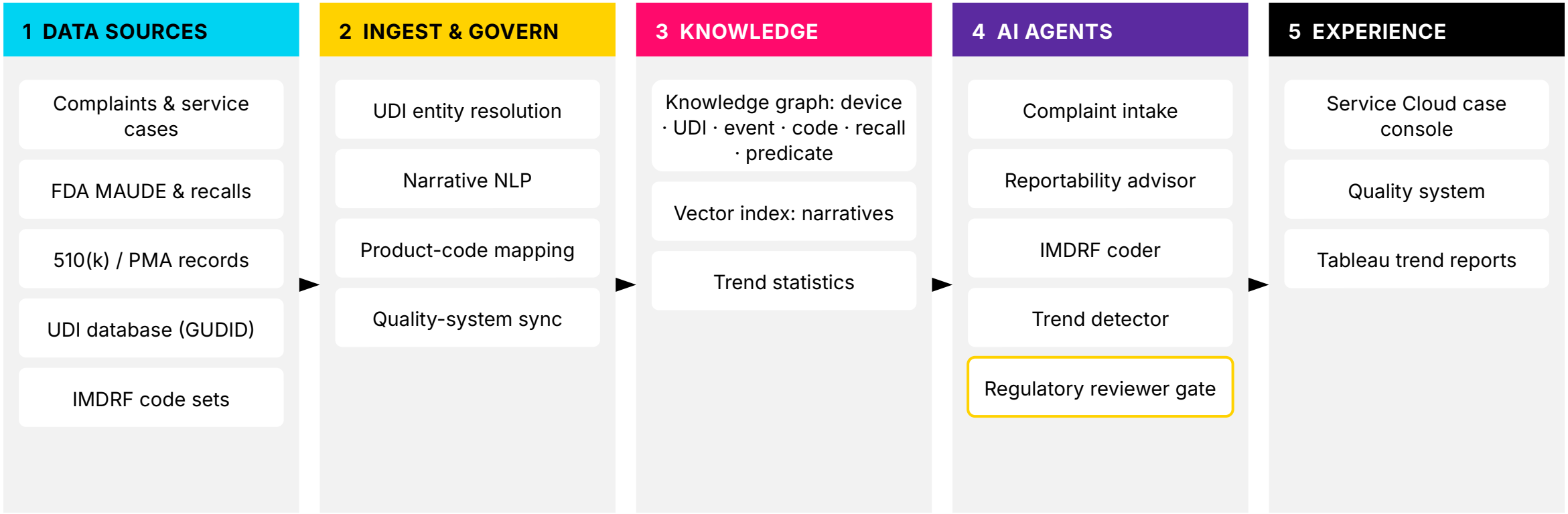
- Intake agent structures complaints from calls, email and service records
- Reportability advisor suggests the FDA / EU MDR decision with cited precedent; a person decides
- Coding agent applies standard adverse-event codes (IMDRF)
- Trend agent watches MAUDE, recalls and internal data for early signals

BUILT ON

Service Cloud | Health Cloud | Tableau

HIGH-LEVEL ARCHITECTURE

How it works: Device complaints & surveillance



GOVERNANCE Guardrails in & out · PHI masking & minimum-data access · Human approval gates · Evaluation set · Audit trail

Runs on any cloud: Google Cloud · AWS · Azure · Salesforce

○ Yellow outline = human in the loop

VALUE & BENEFITS

Faster, more consistent complaint handling and earlier safety signals.

↓

Complaint cycle time

Pilot target

0

late FDA reports (goal)

Pilot target

~59%

agreement for an unaided AI model with human coding, which is why graph + human review matter

Source: 2026 MAUDE study (preprint)

↑

Trend detection lead time

Pilot target

Business

Lower cost of quality, fewer surprise recalls

Clinical

Patient-safety issues found earlier

IT & data

One device graph linking UDI, complaints and recalls

Compliance

21 CFR 803, EU MDR vigilance and audit trail

Remote patient monitoring & connected devices

THE PROBLEM

Connected devices create a flood of readings. Care teams drown in alerts, and billing rules (days of data, minutes of care) are tracked by hand.

Who feels it: Virtual care and RPM programs, care managers, device and RPM vendors

- Alert fatigue hides the patients who need help
- Device data isn't in the care team's workflow
- Revenue lost when billing requirements aren't tracked

16 days

of readings per 30-day period required for the core RPM billing code (CMS adds shorter-period codes for 2026)

Source: CMS Physician Fee Schedule

OUR SOLUTION

Specialist AI agents, grounded in trusted data, with people in control

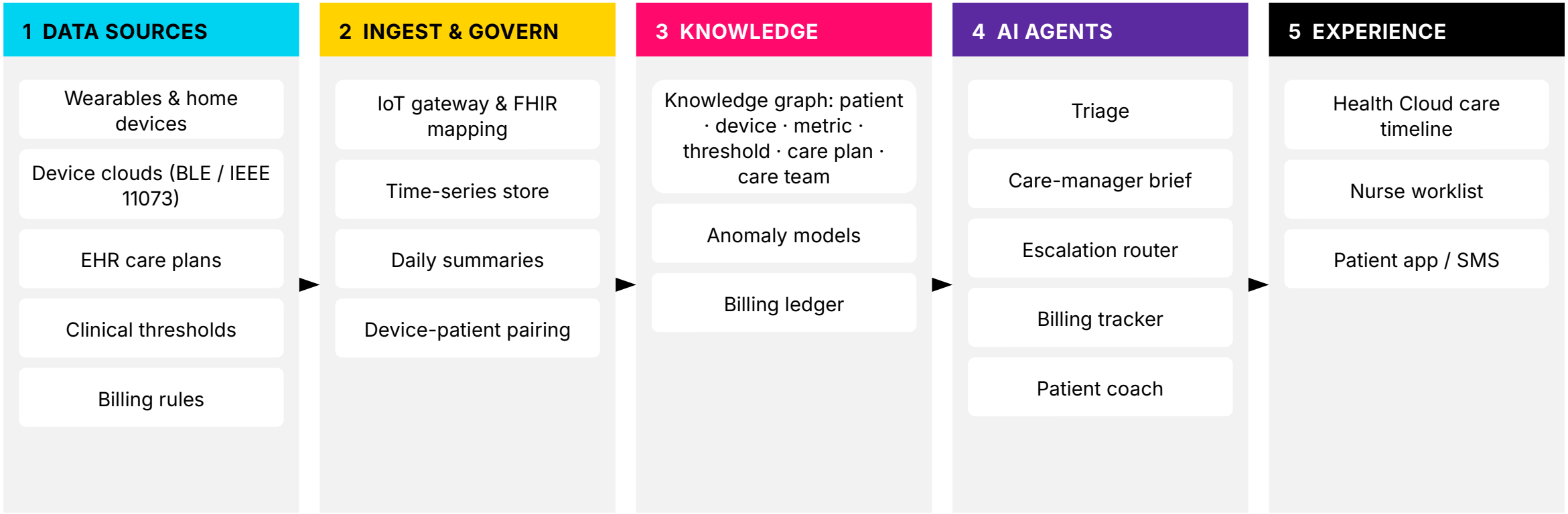
- Devices stream into a standard FHIR format; only daily summaries go into the graph
- Triage uses clinical rules and anomaly models to decide; AI writes the explanation
- Care-manager daily brief and escalation routing to the right role
- Billing tracker and patient coaching nudges keep programs on track

BUILT ON

Health Cloud | MuleSoft / Data Cloud | Agentforce | Marketing Cloud

HIGH-LEVEL ARCHITECTURE

How it works: Remote patient monitoring



GOVERNANCE Guardrails in & out · PHI masking & minimum-data access · Human approval gates · Evaluation set · Audit trail

Runs on any cloud: Google Cloud · AWS · Azure · Salesforce

○ Yellow outline = human in the loop

VALUE & BENEFITS

Fewer, better alerts and a remote-monitoring program that pays for itself.



Alerts per nurse per day

Pilot target



Time to escalate a high-risk reading

Pilot target



Patients meeting billing requirements

Pilot target



30-day readmissions in enrolled cohorts

Pilot target

Business

Billable, scalable virtual-care revenue

Clinical

Earlier intervention, fewer readmissions

IT & data

Standards-based device integration (FHIR)

Compliance

Clinician decides; FDA decision-support guidance respected

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05 MANUFACTURING & SUPPLY CHAIN

A · GMP deviation & CAPA investigation copilot

B · Drug shortage early warning & supply resilience

Who buys: Pharma and biotech manufacturing, CDMOs, distributors, hospital pharmacy

GMP deviation & CAPA investigation copilot

THE PROBLEM

Deviation investigations and corrective actions (CAPAs) take weeks. Similar past events are buried in quality records, and repeat findings lead to FDA observations and warning letters.

Who feels it: QA investigators, site quality heads, manufacturing engineers

- Investigations start from a blank page
- Repeat deviations because root causes aren't connected
- Overdue CAPAs and inspection risk

40%

reduction in deviation closure time reported in industry GenAI case studies

Source: McKinsey (vendor-reported)

OUR SOLUTION

Specialist AI agents, grounded in trusted data, with people in control

- Triage agent sets criticality and routes the deviation
- Similar-event finder links batch, equipment, material lot and past root causes
- Root-cause drafter writes the 5-Whys / fishbone with citations; QA decides
- CAPA checker tracks effectiveness and matches FDA 483 themes

BUILT ON

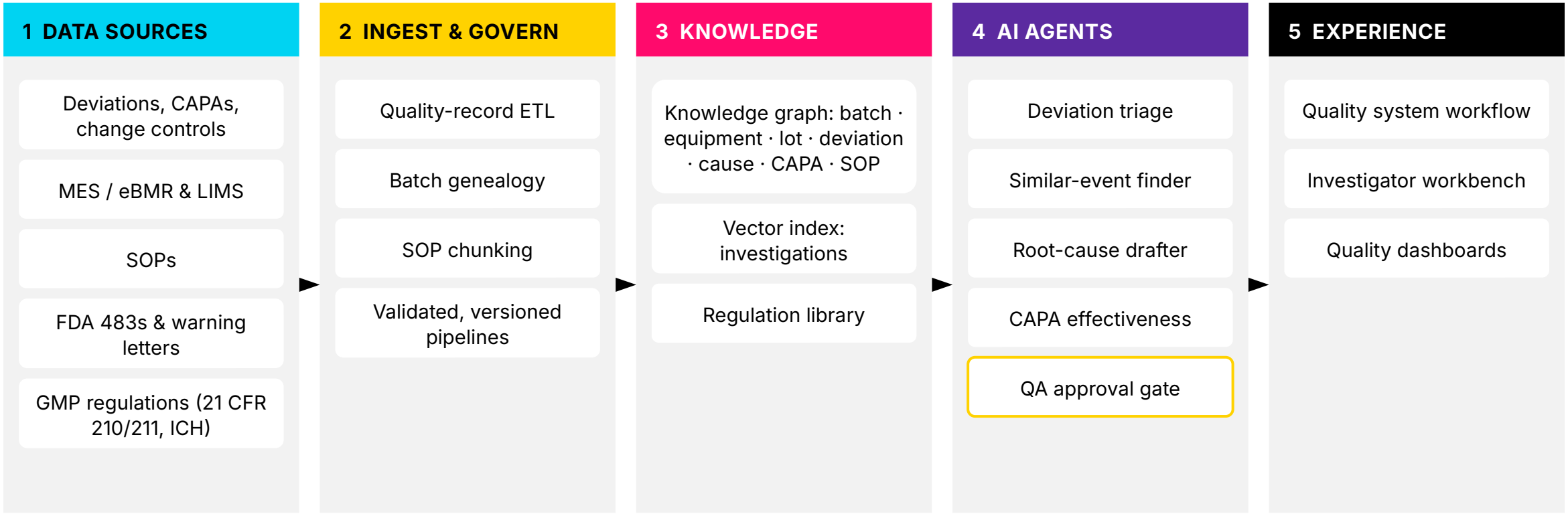
Quality system (e.g. Veeva Vault QMS)

MES / LIMS integration

Any cloud

HIGH-LEVEL ARCHITECTURE

How it works: Deviation & CAPA copilot



GOVERNANCE Guardrails in & out · PHI masking & minimum-data access · Human approval gates · Evaluation set · Audit trail

Runs on any cloud: Google Cloud · AWS · Azure · Salesforce

○ Yellow outline = human in the loop

VALUE & BENEFITS

Close investigations faster and stop repeat deviations, with QA always in control.

40%

faster deviation closure reported in industry case studies

Source: McKinsey (vendor-reported)



Repeat deviations

Pilot target



Overdue CAPAs

Pilot target

100%

of AI drafts approved by qualified QA (Part 11 audit trail)

Design guarantee

Business

More right-first-time batches, less lost product

Clinical

Consistent product quality for patients

IT & data

Validated, versioned AI aligned to GAMP 5 / CSA

Compliance

Inspection-ready; aligned to Part 11, Annex 11 and draft Annex 22 (AI)

Drug shortage early warning & supply resilience

THE PROBLEM

Hospitals and manufacturers learn about shortages late. Finding a safe substitute and re-planning supply is manual and slow.

Who feels it: Hospital pharmacy and supply chain, manufacturer supply planning, distributors

- Little warning before stock-outs
- Single-source suppliers and facilities are hidden risks
- Therapeutic substitution decided under pressure

323

active US drug shortages in Q1 2024, an all-time high

Source: ASHP

OUR SOLUTION

Specialist AI agents, grounded in trusted data, with people in control

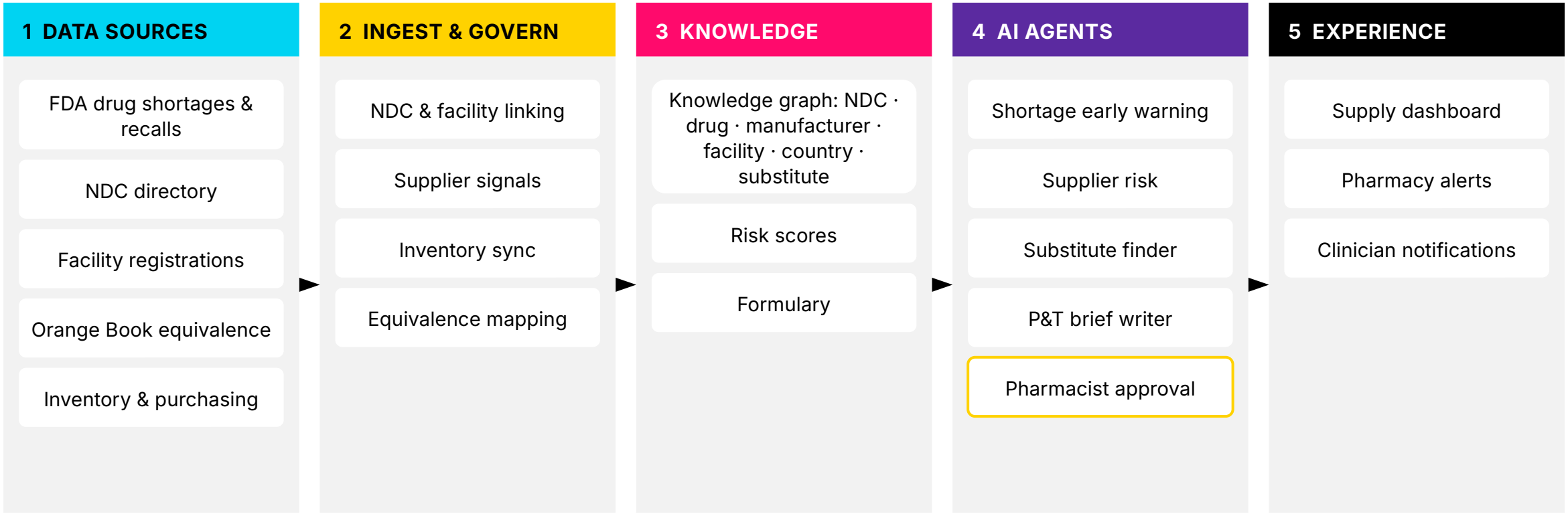
- Early-warning agent watches FDA shortages, recalls, warning letters and supplier signals
- Risk map links each product to manufacturer, facility and country
- Substitute finder checks equivalence, formulary and label before suggesting options
- Brief writer prepares Pharmacy & Therapeutics committee and clinician alerts

BUILT ON

Any cloud | Tableau | Health Cloud / Service Cloud alerts

HIGH-LEVEL ARCHITECTURE

How it works: Shortage early warning



GOVERNANCE Guardrails in & out · PHI masking & minimum-data access · Human approval gates · Evaluation set · Audit trail

Runs on any cloud: Google Cloud · AWS · Azure · Salesforce

○ Yellow outline = human in the loop

VALUE & BENEFITS

See shortages coming and keep patients on safe therapy.



Days of warning before stock-out

Pilot target



Time to approve a substitute

Pilot target



Patients affected by stock-outs

Pilot target

323

active US shortages at the Q1 2024 peak

Source: ASHP

Business

Less emergency buying, lower waste

Clinical

Continuity of care during shortages

IT & data

Product-to-facility graph reused for supplier risk

Compliance

Pharmacist approves every substitution

How we engage: from idea to production

Ideation workshop
2 WEEKS

Pick the use case, map data and stakeholders, agree success metrics.

Proof of concept
6–8 WEEKS

Working agents on public and synthetic data, HLD signed off, demo to sponsors.

Pilot
~3 MONTHS

Real users, governed client data, measured against the agreed KPIs.

Scale
ONGOING

Roll out across sites and brands, add the next domain on the same platform.

Next step: pick one case study and run a 2-week ideation workshop with the client.

Sales cheat sheet: opportunity triggers

Domain	Listen for...	Who to talk to	Case studies to bring
Care Engagement	Care gaps, low screening rates, no-shows, outreach that doesn't convert, patient leakage	CMO / CNO, VP Population Health, Patient Access, Marketing	Care-gap closure & journeys · Access & financial navigation
Life Sciences	Launch planning, HCP engagement, slow MLR review, trial enrollment delays, site selection	Brand / commercial leads, Medical Affairs, Clinical Operations	HCP engagement & MLR pre-review · Trial feasibility & recruitment
Pharmacy & Drug	Medication errors, prior-auth backlogs, CMS-0057-F, adherence, adverse-event capture	Chief Pharmacy Officer, VP Patient Services, Drug Safety	Med reconciliation & interaction safety · Specialty Rx hub & prior auth
Medical Devices & IoT	Complaint backlogs, FDA / EU MDR reporting, recalls, remote monitoring programs, alert fatigue	VP Quality / Regulatory, Post-Market Surveillance, Virtual Care leaders	Device complaints & surveillance · Remote patient monitoring
Manufacturing & Supply Chain	Deviation backlogs, FDA 483s / warning letters, CAPA overdue, drug shortages, supplier risk	VP Quality / QA, Site Heads, Supply Chain, Hospital Pharmacy Directors	Deviation & CAPA copilot · Shortage early warning



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